Memorial General Hospital - Department of Inpatient Medicine

CLINICAL DOCUMENTATION — NARRATIVE SURGICAL DISCHARGE SUMMARY V1

Patient: Charles Anderson | **MRN:** PAT-60013 | **DOB:** 1959-09-28 | **Gender:** Male | **Doc ID:** NARRATIVE_0013
Encounter Date: 2025-05-08 | **Discharge/Note Date:** 2025-05-12 | **Provider:** Dr. Allison Cameron, MD

PREOPERATIVE DIAGNOSIS & PROCEDURE

The patient underwent uncomplicated arthroscopic knee meniscectomy and chondroplasty under general endotracheal anesthesia.

HISTORY OF PRESENT ILLNESS

The patient is an adult who presented electively for surgical intervention following conservative management failure. Pre-admission testing confirmed that preoperative total cholesterol was 168.2 mg/dL. Preoperative metabolic screening showed a baseline platelet count of 244.9 10*3/uL. The patient was cleared from a cardiopulmonary standpoint and taken to the operative suite.

POSTOPERATIVE HOSPITAL COURSE

The patient was extubated in the operating room and transferred to the post-anesthesia care unit in stable condition. Postoperative recovery was uneventful. For pain and infection prophylaxis, the patient was prescribed Lisinopril 40 mg PO every morning. On postoperative day one, postoperative alanine aminotransferase (alt) remained stable at 79.3 U/L (HIGH). Surgical incisions were clean, intact, and without erythema or purulent drainage.

POST-DISCHARGE MEDICATIONS

Discharge medications: The patient will take Metformin 500 mg PO twice daily with meals, and continue baseline Atorvastatin 80 mg orally every evening. All prescriptions were transmitted electronically to the patient's pharmacy.

ACTIVITY & WOUND CARE PLAN

Keep dressing clean and dry. No heavy lifting greater than 10 pounds for two weeks. Surgical clinic follow-up scheduled in 10 days.